Diagnostics waiting over six weeks

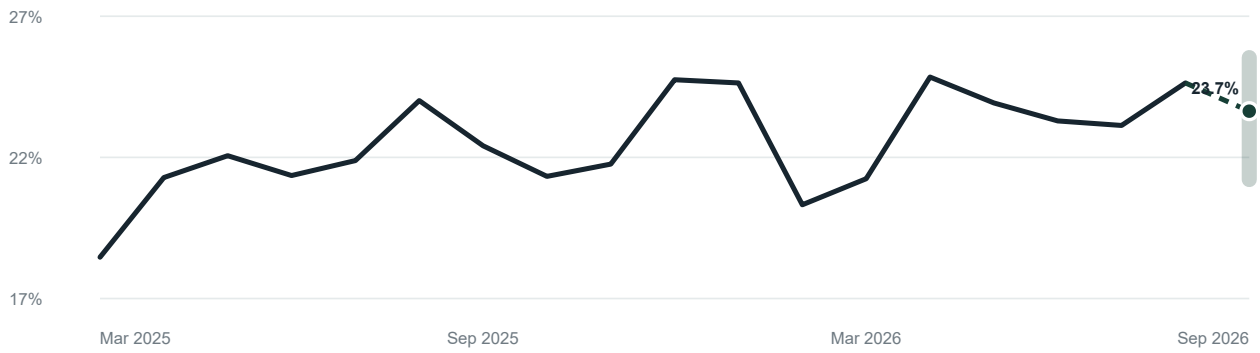
Forecast for September 2026 · data through August 2026

The latest Diagnostics waiting over six weeks result was 24.7%. For the next release, it is expected to improve to 23.7%. The model's expected improvement is mainly due to the usual month-to-month seasonal movement; the underlying trend is broadly flat. The current projection is off course for 14% or less by March 2027. Across providers, sustained favourable signals outnumber adverse ones (50 to 41).

LATEST · AUGUST 2026	FORECAST · SEPTEMBER 2026	MAR 2027 TARGET OUTLOOK
24.7%	23.7%	Projected 21.1%
0.5pp below forecast	80% expected range 21.2%–25.6%	Target ≤14%
		8% Model-implied chance

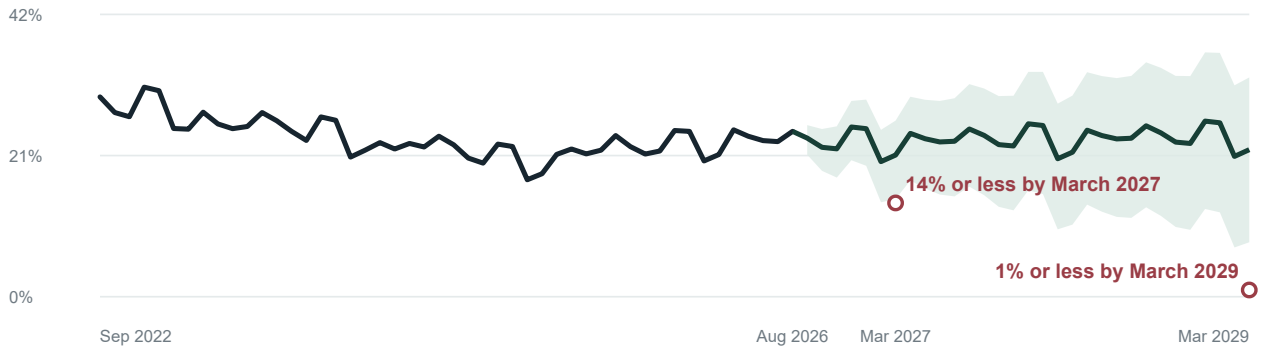
Recent performance and next release

The vertical band is the 80% expected range.



Longer-term outlook and milestones

Descriptive projection; not forced to meet targets.



Benchmark: 1% operational standard. 7.1pp above milestone

Data: NHS England diagnostic waiting times and activity. **Targets:** Medium Term Planning Framework national target · Medium Term Planning Framework final target

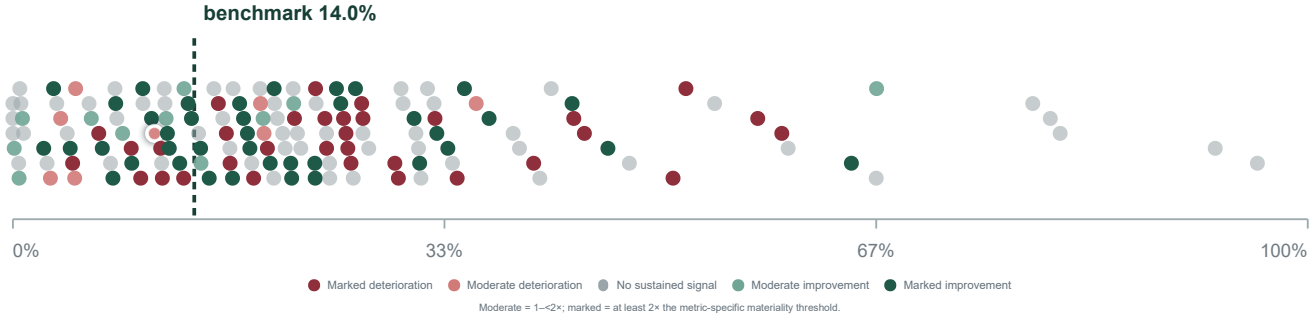
Method: Backtest-weighted ensemble of current-level seasonal movement, damped annual drift and a damped recent seasonal trend; at least 2 components are required. The final fit uses 248 consecutive months. Accuracy is tested on 36 rolling one-month-ahead forecasts. The 80% range uses empirical historical error quantiles from 36 forecast errors. Weights use inverse rolling RMSE and are shrunk toward equal weights. The pre-release commentary compares the current-level seasonal component with the combined forecast. Provider distribution and trajectory watch are shown on page 2.

Diagnostics waiting over six weeks

Distribution and trajectory watch · August 2026

Provider distribution

Latest reported performance across providers. This is a comparison of current levels, not a measure of whether each provider is improving or deteriorating.



Moderate = 1–<2×; marked = at least 2× the metric-specific materiality threshold.

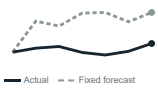
Trajectory watch

Six actual months are compared with one forecast path fixed before that window. Both lines begin at the same observed value. A low performer can be improving, while a high performer can be deteriorating. Status: Actual performance is compared with the monthly path forecast six months earlier. The displayed figure is the latest actual minus the fixed forecast for that same month; the six-month window checks persistence.

142	50	41	51
providers assessed	sustained favourable	sustained adverse	without an active signal

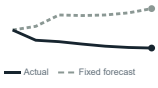
Largest sustained favourable moves

West London NHS Trust
Latest 64.8% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



34.7pp
LATEST MONTH: BELOW FORECAST

Croydon Health Services NHS Trust
Latest 3.1% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



29.7pp
LATEST MONTH: BELOW FORECAST

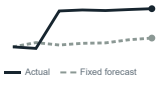
Royal Surrey NHS Foundation Trust
Latest 12.1% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



29.4pp
LATEST MONTH: BELOW FORECAST

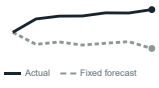
Largest sustained adverse moves

Royal Papworth Hospital NHS Foundation Trust
Latest 59.4% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



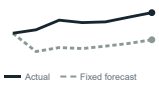
40.3pp
LATEST MONTH: ABOVE FORECAST

Dorset Healthcare University NHS Foundation Trust
Latest 34.3% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



27.4pp
LATEST MONTH: ABOVE FORECAST

East Kent Hospitals University NHS Foundation Trust
Latest 52.0% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



21.2pp
LATEST MONTH: ABOVE FORECAST

Trajectory-watch rule. Six actual months are compared with the corresponding path fixed before the six-month window. The displayed gap is the latest actual minus the fixed forecast for that same month. A signal requires a material latest-month gap and at least five of six months in that direction. These are screening prompts—not league tables, causal estimates or judgements of provider quality.